

EXECUTIVE ANALYTICS BRIEFING

Indian Healthcare Analytics

What 64.97K patient records reveal
about burden, access, risk, and
treatment performance.

National Healthcare Analytics Review
Multi-State Longitudinal Study · 2024–2026

Synthesized from 100K clinical encounters across 10 Indian
states





OPERATING SCOPE & COHORT BASELINE

The dashboard turns 100K visits into one operating picture

Connect patient mix, diagnosis, clinical markers, access, and outcomes in one filterable model.

100K

Total Visits

10 Indian states ·
2024–2026

64.9K

Active Patients

44.4

Average Age

1.06 male-to-
female ratio

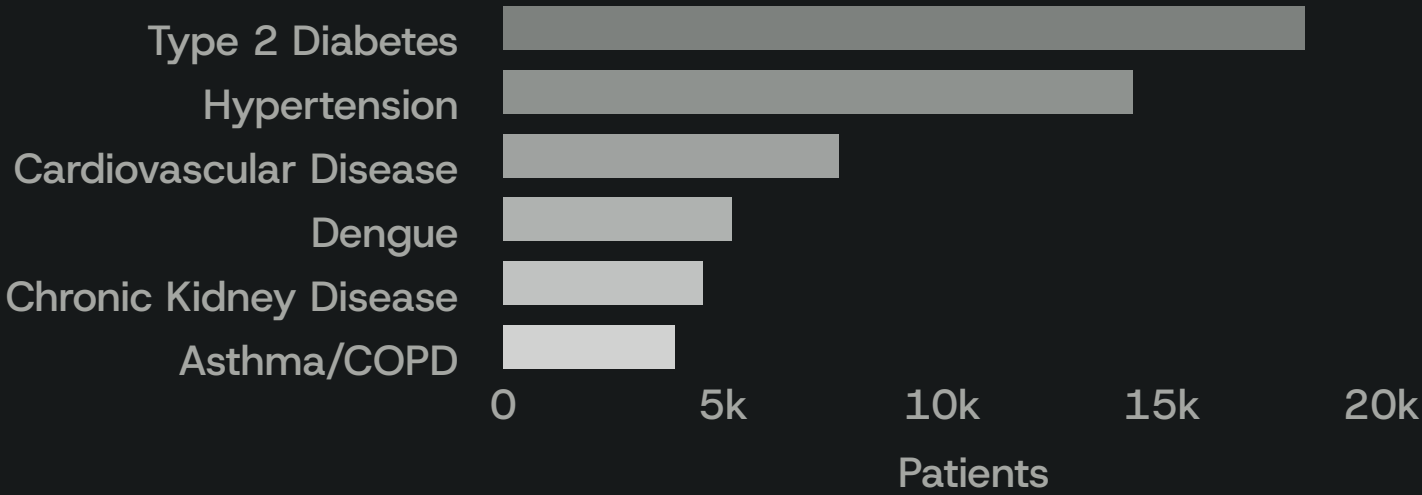
Data range: 27 May 2024 to 26 May 2026

Made with GAMMA

Diabetes and hypertension carry half the visible diagnosis burden

Type 2 diabetes and hypertension represent 32.6K patients, concentrating care demands before acute interventions begin.

Diagnosis



Primary diagnosis classification · 64.97K patient dashboard cohort

Key Concentration

50.2%

of all documented visits stem from Type 2 diabetes (18.26K) and hypertension (14.34K) combined.

Cardiovascular conditions follow closely at 7.65K, creating significant multi-morbidity risk.

GEOGRAPHIC DISPERSION

Patient volume is concentrated in the south and west

Tamil Nadu, Kerala, and Karnataka account for nearly 45% of total active registry encounters.

TIER 1 LEADER

11.69K

Tamil Nadu

Highest regional volume, anchoring southern tertiary treatment corridors.

HIGH DENSITY

9.75K

Kerala

High diagnostic reporting and follow-up density across public and private clinics.

URBAN HUB

7.81K

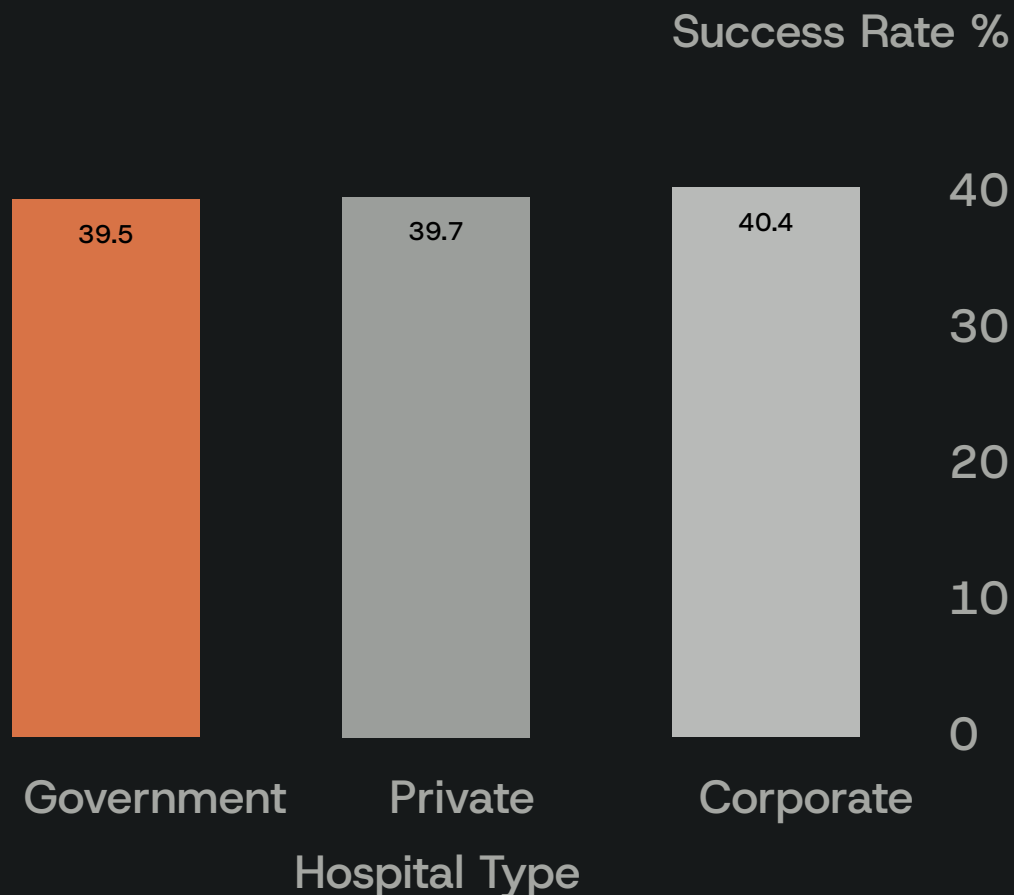
Karnataka

Major specialty hub absorbing complex chronic cases from neighboring districts.

Next tier: Andhra Pradesh 6.49K and Telangana 5.82K.

Implication: regional capacity planning should start where patient density and risk intersect.

Treatment outcomes are balanced, but success is not yet dominant



OUTCOME BREAKDOWN

39.7%

Improved or recovered

Worsened: 20.0%; **referred:** 19.98%. Remaining outcomes are broadly stable or recovered.

Hospital-type pattern: corporate hospitals lead at 40.4% success, just ahead of private at 39.7% and government at 39.5%.

Access is the clearest equity gap

Nearly two-thirds of all patients enter care without health coverage, placing the heaviest treatment load on public infrastructure.

65.0%

Uninsured Patients

Only 35.0% carry active insurance coverage across the registry.

44.6%

Government Share

Public hospitals handle 44.6%, private 40.1%, and corporate 15.2%.

Management Challenge: Pinpointing where uninsured, low-socioeconomic populations face disproportionate risks of high-cost chronic complications.



Clinical markers point to a meaningful risk-management opportunity

Glucose

127.7

mg/dL

HbA1c

6.4%

Cholesterol

216

mg/dL

BMI

24.6

kg/m²

73.48% of Type 2 diabetes patients are shown as uncontrolled.

683 patients in the treatment-outcomes view.

Prioritize regional prevention and targeted follow-up

Four synchronized interventions designed to shift performance from reactive inpatient care to managed preventive health.

01	02	03	04
Targeted Pathways Focus prevention and chronic-care protocols directly on diabetes, hypertension, and cardiovascular disease.	Coverage Expansion Close the 65% insurance gap through region- and socioeconomic-specific outreach and enrollment drives.	Active Risk Triage Deploy predictive composite risk scoring to trigger early proactive outreach for patients with multi-marker elevation.	Clinical Governance Institute monthly operating reviews tracking success, worsening, referral, and seasonal patient volume.

From dashboard to action

Phase 1

Refresh the cleaned dataset and validate the 14 DAX measures.

Phase 2

Publish the seven dashboard pages with shared slicers for year, region, hospital type, gender, socioeconomic status, and diagnosis.

Phase 3

Add commentary, scheduled refresh, and stakeholder sharing in Power BI Service.

